

OXYLA 10 IU/ml INJECTION
Oxytocin

Product Description

Clear solution, colourless

Composition

Each ml of injection contains oxytocin 10 IU/ml.

Pharmacodynamics

Oxyla is a synthetic octapeptide identical with oxytocin, a hormone released by the posterior lobe of the pituitary. Being wholly synthetic, it does not contain vasopressin and has a constant and reliable effect. Oxytocin stimulates the smooth muscle of the uterus, producing rhythmic contractions. Synthetic oxytocin has only very slight pressor and antidiuretic activity (absence of vasopressin).

Pharmacokinetics

Oxytocin is distributed throughout the extracellular fluid. Small amounts of oxytocin probably reach the fetal circulation. Oxytocin has a plasma half-life about 3-5 minutes. Most of the drug is rapidly destroyed in the liver and kidneys. Oxytocinase, a circulating enzymes produced early in pregnancy, is also capable of inactivating the polypeptide. Only small amounts of oxytocin are excreted in urine unchanged.

Indications

Induction of labour for medical reasons (e.g. postmaturity, pre-mature rupture of the membranes, preeclampsia). Caesarean section following delivery of the infant. Prevention and treatment of postpartum haemorrhage associated with uterine atony.

Recommended Dosage

Induction or enhancement of labour

Oxytoxin should be administered as an i.v. drip infusion or, preferably, by means of a variable-speed infusion pump. For drip infusion it is recommended that 5 IU of Oxytoxin be added to 500 mL of a physiological electrolyte solution (such as sodium chloride 0.9%). For patients in whom infusion of sodium chloride must be avoided, 5% dextrose solution may be used as the diluent (see section SPECIAL WARNINGS AND PRECAUTIONS FOR USE). To ensure even mixing, the bottle or bag must be turned upside down several times before use.

The initial infusion rate should be set at 1 to 4 milliunits/minute (2 to 8 drops/minute). It may be increased gradually at intervals not shorter than 20 minutes until a contraction pattern similar to that of normal labour is established. In pregnancy near term, this can often be achieved with an infusion of less than 10 milliunits/minute (20 drops/minute), and the recommended maximum rate is 20 milliunits/minute (40 drops/minute). In the unusual event of higher rates being required, as may occur in the management of foetal death in utero or for induction of labour at an earlier stage of pregnancy when the uterus is less sensitive to oxytocin, it is advisable to use a more concentrated Oxytoxin solution, e.g. 10 IU in 500 mL.

When using a motor-driven infusion pump which delivers smaller volumes than those given by drip infusion, the concentration suitable for infusion within the recommended dosage range must be calculated according to the specifications of the pump.

The frequency, strength and duration of contractions and also the foetal heart rate must be carefully monitored throughout the infusion. Once an adequate level of uterine activity is attained, the infusion rate can often be reduced. In the event of uterine hyperactivity and/or foetal distress, the infusion must be discontinued immediately.

If, in women who are at term or near term, regular contractions are not established after the infusion of a total amount of 5 IU, it is recommended that the attempt to induce labour should be terminated; it may

be repeated on the following day, starting again from a rate of 1 to 4 milliunits/minute.

Note:

Inadvertent paravenous infusion of oxytocin is not harmful.

Caesarean section

5 IU by i.v. infusion (5 IU diluted in physiological electrolyte solution and administered as an i.v. drip infusion or, preferably, by means of a variable-speed infusion pump over 5 minutes) immediately after delivery.

Prevention of postpartum uterine haemorrhage

The usual dose is 5 IU by i.v. infusion (5 IU diluted in physiological electrolyte solution and administered as an i.v. drip infusion or, preferably, by means of a variable-speed infusion pump over 5 minutes) or 5 to 10 IU i.m. after delivery of the placenta. In women given Oxytoxin for induction or enhancement of labour, the infusion should be continued at an increased rate during the third stage of labour and for the next few hours thereafter.

Treatment of postpartum uterine haemorrhage

5 IU by i.v. infusion (5 IU diluted in physiological electrolyte solution and administered as an i.v. drip infusion or, preferably, by means of a variable-speed infusion pump over 5 minutes), or 5 to 10 IU i.m., followed in severe cases by intravenous infusion of a solution containing 5 to 20 IU of oxytocin in 500 mL of an electrolyte-containing diluent, run at the rate necessary to control uterine atony.

Incompatibilities

Oxytocin should not be infused via the same apparatus as blood or plasma, because the peptide linkages are rapidly inactivated by oxytocin-inactivating enzymes. Oxytocin is incompatible with solutions containing sodium metabisulphite as a stabiliser.

Route of Administration

Intravenous & Intramuscular

Contraindications

Hypersensitivity to the drug.
Hypertonic uterine contractions; fetal distress (unless delivery is imminent).
Severe toxæmia; predisposing factors for amniotic fluid embolism (e.g. intrauterine death, abruptio placentae).
All conditions in which, for fetal or maternal reasons, spontaneous labour should be avoided and/or vaginal delivery is contraindicated, e.g.: Cephalopelvic disproportion, abnormal presentation; placenta prævia, vasa prævia, abruptio placentae, torsion or prolapse of the umbilical cord, overdistension or reduced resistance of the uterus with risk of rupture (e.g. multiple pregnancy, hydramnios, multiparity greater than four, multiparity in older women, history of major uterine surgery, including classic Caesarean section).
Oxytocin should not be infused over long periods in women with uterine inertia who do not respond adequately to oxytocin or in patients with severe preeclampsia or cardiovascular disturbances.

Warnings and Precautions

The induction of labour by means of oxytocin should be attempted only when strictly indicated for medical reasons. It should only be performed under hospital conditions and qualified medical supervision. Oxyla (Oxytocin), when given for induction or enhancement of labour, must be used only as IV drip infusion and never by IV bolus injection. Careful monitoring is important (fetal heart rate, if possible ocometry, blood pressure), so that dosage may be adjusted to individual response.
Oxytocin should not be given parenterally at the same time as oxytocin buccal tablets or nasal spray.
In patients with cardiovascular disorders, the infusion volume should be kept low by using a more concentrated solution.

To bring on (induce) or maintain labour, Oxyla is given by intravenous (drip). The speed of the infusion is set to maintain a pattern of contractions similar to normal labour. During the infusion, both you and your baby will be closely monitored to prevent complications. If Oxyla is needed at delivery or to prevent excessive bleeding, it can also be given intramuscularly (into a muscle).

Interactions with Other Medicaments

Prostaglandins may potentiate the effect of oxytocin and very careful monitoring is therefore recommended in cases of concomitant administration. Inhalation anesthetics and vasoconstrictor agents should not be used with Oxyla.

Pregnancy and Lactation

Women of childbearing potential

Not applicable for oxytocin because of the targeted indications.

Pregnancy

No standard teratogenicity and reproductive performance studies with oxytocin are available. Based on the wide experience with this drug and its chemical structure and pharmacological properties, it is not expected to present a risk of foetal abnormalities when used as indicated.

Breast-feeding

Oxytocin may be found in small quantities in mother's breast milk. However, oxytocin is not expected to cause harmful effects in the newborn because it passes into the alimentary tract where it undergoes rapid inactivation.

Fertility

Not applicable for oxytocin because of the targeted indications.

Side Effects

When oxytocin is used by i.v. infusion for the induction or enhancement of labour, its administration at excessive doses results in uterine overstimulation which may cause foetal distress, asphyxia and death, or may lead to hypertonicity, tetanic contractions or rupture of the uterus.

Rapid i.v. bolus injection of oxytocin at doses amounting to several IU may result in acute short-lasting hypotension accompanied with flushing and reflex tachycardia. These rapid hemodynamic changes may result in myocardial ischemia, particularly in patients with pre-existing cardiovascular disease. Rapid i.v. bolus injection of oxytocin at doses amounting to several IU may also lead to QTc prolongation.

In rare circumstances, the pharmacological induction of labour using uterotonic agents including oxytocin increases the risk of postpartum disseminated intravascular coagulation.

Water intoxication

Water intoxication associated with maternal and neonatal hyponatraemia has been reported in cases where high doses of oxytocin have been administered together with large amounts of electrolyte-free fluid over a prolonged period of time.

The combined antidiuretic effect of oxytocin and the i.v. fluid administration may cause fluid overload leading to a hemodynamic form of acute pulmonary oedema without hyponatraemia

Symptoms and Treatment of Overdose

Oxytocin overdosage may give rise to following complications: fetal distress, e.g. slowing of fetal heart rate, meconium staining of the amniotic fluid, fetal asphyxia. Uterine hypertonicity, tetanic contraction or rupture.

Treatment:

The infusion should be immediately discontinued and the infusion line occluded at the point of entry into the skin. Oxygen should be administered to the mother. In the rare event that exaggerated contractions persist after stopping the infusion, a selective 2-adrenergic stimulant (such as Ritrodrene) can be used

parenterally to achieve uterine relaxation, in order to relieve fetal distress. The mother may be placed in the left lateral position.

Storage Conditions

Store at cold room (2 - 8°C).

After reconstitution: Store below 30°C for 24 hours.

Dosage Forms and Packaging Available

Box of 10 ampoules @ 1 ml.

Box of 100 ampoules @ 1 ml.

Shelf-life

24 months

Registration Number

OXYLA 10 IU/ml INJECTION – MAL15105027AZ

Manufacturer

PT. Novell Pharmaceutical Laboratories

Jl. Wanaherang No. 35, Tlajung Udik,

Gunung Putri, Bogor 16962 – Indonesia

Product Registration Holder

Averroes Pharmaceuticals Sdn. Bhd.

03-08-01 & 03-09-01, Block 3, Presint Alami, Worldwide Business Center II,

Persiaran Akuatik, Seksyen 13,

40100 Shah Alam, Selangor Malaysia

Tel: +603 5511 1433

Fax: +603 5511 1431

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Manufactured by:

NOVELL
PHARMACEUTICAL
LABORATORIES
BOGOR-INDONESIA



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